

PACIFIC DIAGNOSTICS LAB

Clinical & Reference Laboratory Services
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CLIA #05D2174981 · CAP#88419-22

Accession PDX-25071815
Report Date: 2025-07-18 14:20 PT
Report Status: FINAL
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PDX25071815

PATIENT		ORDERING PROVIDER		SPECIMEN	
Name	CHEN, MARGARET	Name	RAO, ANJALI MD	Type	Serum, SST
DOB	1967-08-14 (57F)	NPI	1659302147	Fasting	Yes (12 hr)
MRN	MRN-2026-04481	Practice	Berkeley Family Medical Grp.	Collected	2025-07-18 07:48 PT
Sex	F	Account	BFMG-022	Received	2025-07-18 12:02 PT
Phone	(510) 555-0148	ICD-10	E78.5; I10; E11.9	Container	Gold-top SST x2
Address	4421 Magnolia Ave, Apt 3B Berkeley, CA 94705	Indication	Hyperlipidemia f/u; HTN; T2DM	Volume	8.0 mL

LIPID PANEL WITH DIRECT LDL		LOINC 57698-3					
TEST		RESULT	FLAG	REFERENCE RANGE		UNITS	METHOD / NOTE
Cholesterol, Total		285	H	Desirable	<200	mg/dL	Enzymatic colorimetric
				Borderline	200-239 /		
HDL Cholesterol		44	L	Female	≥50	mg/dL	Direct, homogeneous
				Optimal	≥60		
LDL Cholesterol, Calculated		195	H	Optimal	<100	mg/dL	Calculated (Friedewald)
				Near optimal	100-129 /		
Triglycerides		220	H	Borderline	130-159 /	mg/dL	Enzymatic, GPO-PAP
				High	160-189		
Non-HDL Cholesterol		241	H	Normal	<150	mg/dL	Calculated
				Borderline	150-199 /		
				High	200-499		
				Goal	<130 (Total - HDL)		

INTERPRETIVE COMMENTS

Multiple lipid abnormalities consistent with newly identified mixed dyslipidemia. LDL-C 195 mg/dL is in the high range (≥160 mg/dL), HDL-C 44 mg/dL is below the female desirable threshold of 50 mg/dL, and Non-HDL-C 241 mg/dL substantially exceeds the <130 mg/dL goal. Triglycerides 220 mg/dL are also in the high range. Per the 2018 ACC/AHA Cholesterol Clinical Practice Guideline, initiation of a moderate- to high-intensity statin is reasonable in patients with diabetes mellitus and additional ASCVD risk factors. Reference ranges per NCEP ATP III; clinical decisions should be individualized.